

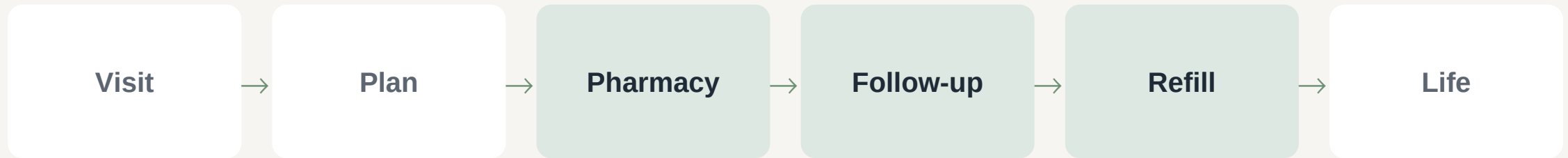
Siya Health®

**We started with patients.
Not software.**

Physician-led ADHD + primary care telehealth · four states (CA · TX · PA · FL)

The problem

**Patients weren't failing to find care.
Care was failing to follow them.**



Everyone optimizes the first visit.

Care actually breaks down at Pharmacy · Follow-up · Refill.

The solution

So we built the missing pieces ourselves.

Physicians → Operators → Systems builders

Multi-state licensure

CA · TX · PA · FL — providers credentialed and compliant in every state we serve.

Offshore clinical operations

Hired, trained, and QA'd from scratch — scheduling, pharmacy coordination, patient follow-up.

Standardized provider workflows

Protocols and SOPs shared across all providers and states — care doesn't depend on memory

Product

The care journey we run today — end to end.

What we deliver today

End-to-end ADHD care, held together by our protocols and clinical judgment.

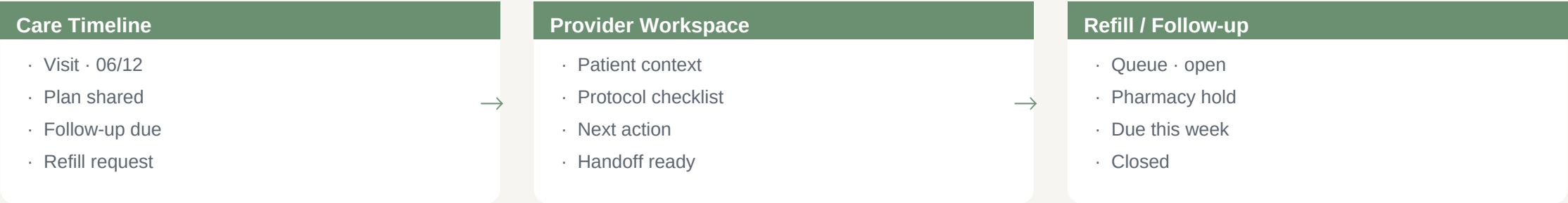


Escalation branch

When care stalls (pharmacy · side effects · missed follow-up · refill risk) → escalate → re-enter Review → Plan → Execute. Continuity is the process.

What we’re building

Concept — in design, informed by workflows already validated in our practice.



Flowchart = care we run today. Wireframe = software encoding that journey (supporting).

Traction

Revenue nearly 3× since February.
Cash-flow positive.

2,000+

Patients treated

1,000+

ADHD evaluations

150+

Recurring members

600+

Reviews · 4.8★

4 states

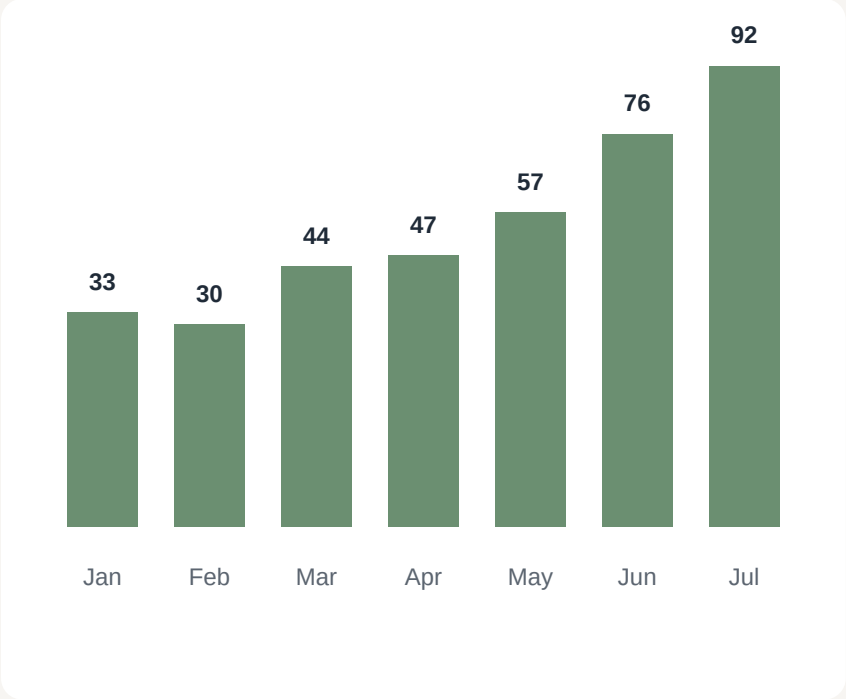
CA · TX · PA · FL

Cash-flow+

Operations

\$378k+ gross 2026 YTD (Jan–Jul) · founder-confirmed books.

Monthly gross revenue — 2026



\$ thousands / month · gross

Where we are, honestly

Our growth engine today is founder-led — and it works.

Proof

Founder-led relationships and networking — on top of a full clinical load — fill all providers' panels, not just the founder's own.

Risk

That is a key-person dependency.
We say so plainly.

Fix

This round funds clinical capacity and redundancy, so proven acquisition runs without routing every relationship through one person.

Proof + risk + fix — not a disclosure buried in an appendix.

Cash-pay, recurring, no insurance risk.

\$149

Initial evaluation

One-time, self-pay

\$79 / mo

Ongoing care — non-controlled

Recurring membership

\$149 / mo

Ongoing care — controlled

Recurring membership

Subscription-style continuity: 150+ recurring members and growing.

Roughly \$600 per patient-year in professional fees (bottom-up, competitor-benchmarked).

Cash-pay means no payer contracts, no reimbursement risk, and revenue from day one.

15.5M diagnosed adults. \$2.9B serviceable today.

Layer	Definition	Size
TAM	U.S. adult ADHD clinical services (professional fees)	~\$10B
SAM	Adult ADHD virtual care (telehealth-willing, cash-pay comps)	~\$2.9B
SOM	Year-3, capacity-derived (~40 clinician FTEs)	~\$16M

Built bottom-up: 15.5M diagnosed U.S. adults (CDC) · ~half already use telehealth for ADHD · ~\$600/patient-year professional fees.

No inflated blended TAM. Full sizing workbook available in diligence.

Competition

Different job. Not a feature checklist.

Episodic telehealth	Optimizes first-visit volume, not continuity
Generic EHR tools	Records history, doesn't guide next action
Point cognitive tools	One-time score, not ongoing care
Traditional psychiatry	Access-limited, waitlist-bound
Siya	Continuity, evaluation through refill — 2,000+ patients

No feature matrix. No “everyone top-right” quadrant. The difference is the job we do.

A process — not a channel wishlist.

01 · Today

Primary engine

Founder-led relationships and networking fill all providers' panels — not just the founder's.

02 · Live test

Texas Search

Paid Search live in Texas; conversion tracking finalized Aug 2, 2026.

03 · Transition

This round's job

Add clinical capacity, then move from founder-led fill to systematic acquisition — reproduce Search TX → CA.

04 · Measurement

No guessing

Full CAC and payback period confirmed by August 30, 2026.

Why now

The playbook is proven. The environment rewards how we practice.

Texas

Early conversions confirmed in Texas — full CAC by Aug 30, 2026.

California

Revenue growing — next market for the same playbook

Regulation

Environment rewards protocolized, auditable care over low-touch mills

Discipline

SOPs, QA, and multi-state compliance already in place

We're raising \$500k to scale a working practice.

01 Clinical capacity & redundancy

More providers and coverage — directly reduces founder key-person risk.

02 Scale proven acquisition

Early conversions confirmed in Texas — full CAC by Aug 30, 2026.

03 Working capital buffer

Operate from strength, not desperation.

Objective: 18–24 months of runway to prove the playbook in a second state and remove key-person dependency.

Founders

Physicians who built the practice before asking for capital.



Dr. Sneh Pandey

Founder · CEO

- Board-certified physician
- 10,000+ patient encounters at Siya Health across primary care, obesity, hospital medicine, and ADHD care
- Built multi-state practice to 2,000+ patients, cash-flow positive, bootstrapped



Dr. Swati Pandey

Co-founder

- Board-certified physician, licensed in Pennsylvania
- Co-founder and clinical advisor — shapes clinical protocol standards across Siya Health

**Years spent learning how to deliver better care.
This round scales what already works.**

Seed round

\$500,000

Clinical capacity & redundancy · Proven acquisition, next state · Working capital

Siya Health®

Let's talk.

Dr. Sneh Pandey · Founder & CEO

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